

1. Administrative & Study Identification

Full Study Title: An Observational Study of Glofitamab in Chinese Adult Participants With 2L Diffuse Large B-Cell Lymphoma **Official Title:** Evaluating Effectiveness and Safety of Glofitamab Based Second-Line Therapy in Chinese Adult Patients With Relapsed or Refractory Diffuse Large B-Cell Lymphoma: A Prospective, Observational, Multicenter, Cohort Study **Short Title/Acronym:** GlofitReal **Posting ID:** 884494576106495872 **Protocol Number:** ML45172 **NCT Number:** NCT07200375 **Trial Status:** RECRUITING **Sponsor Name:** Roche (Hoffmann-La Roche) **Investigational Product:** Glofitamab (Columvi)-based therapy **Phase of Development:** Not Applicable **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To investigate how well glofitamab-based therapy works and how safe it is in Chinese adult participants with relapsed or refractory diffuse large B-cell lymphoma (R/R DLBCL). **Secondary Objectives:** To evaluate the Complete Response Rate (CRR), Overall Response Rate (ORR), Disease Control Rate (DCR), and comprehensively assess the safety profile and incidence of adverse events in routine clinical practice.

2.2 Background & Rationale To address the evidence gap regarding the real-world application of glofitamab (Columvi) in Chinese populations with 2L R/R DLBCL. While clinical trials demonstrate the efficacy of glofitamab, prospective, observational, multicenter cohort data is necessary to understand how the therapy works, how safe it is, and how it is utilized outside of controlled clinical environments for treating adult patients.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Observational **Control Type:** Single-arm (Real-world cohort) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration **Target \$N\$:** 300 participants **Number of Sites:** Approximately 12 locations in China (including Henan Cancer Hospital, Guangdong Provincial People's Hospital, Peking University Third Hospital, etc.) **Estimated Study Start:** September 2025 **Estimated Primary Completion:** Late 2027 (Estimated based on standard observational timeframes)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Histologically confirmed relapsed or refractory (R/R) Diffuse Large B-Cell Lymphoma (DLBCL) after one line of systemic therapy. 3. Will be treated with Glofit-based regimen (known as being recommended and having the intention to be treated with Glofitamab at the time of signing ICF) or have initiated Glofit-based regimen treatment within three months (90 days) prior to enrollment.

4.2 Exclusion Criteria 1. Participants who currently participate in or plan to participate in any interventional clinical trial. 2. Lacking necessary clinical data or inability to be followed up per standard clinical practice. 3. Other protocol-defined real-world study exclusions.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Glofitamab (Columvi) administered as per standard prescribing information and treating physician's clinical judgment. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Determined by the treating physician based on disease progression, patient tolerability, and routine clinical practice.

5.2 Concomitant & Prohibited Therapy Permitted: Routine standard-of-care medications, supportive care, and prophylactic treatments for cytokine release syndrome (CRS) per local guidelines. **Prohibited:** Concurrent experimental or investigational systemic anticancer therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening/Enrollment	Day -14 to 0	Informed Consent, Medical History, Confirmation of 2L DLBCL status
Baseline	Day 0	Initial Glofitamab administration data collection
Routine Visits	As per standard care	Tumor response assessments, Safety/AE reporting, Survival tracking
End of Study	Follow-up End	Final survival status, treatment discontinuation reasons

(Note: As an observational study, visit cadence strictly follows local routine clinical practice rather than a mandated trial protocol).

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: 1. real-world Progression-Free Survival (rwPFS). **Measurement Method:** Evaluated through routine

clinical assessments, medical chart abstractions, and local radiology reviews.

7.2 Secondary & Exploratory Endpoints 1. Complete Response Rate (CRR) 2. Overall Response Rate (ORR) 3. Disease Control Rate (DCR) Incidence, severity, and management of Adverse Events (AEs), specifically Cytokine Release Syndrome (CRS) and neurologic toxicities.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Collected continuously from patient medical records during routine oncology care visits.

Serious Adverse Events (SAEs): Reported according to local post-marketing pharmacovigilance and observational registry guidelines. **Data Monitoring Committee (DMC):** Managed via the sponsor's internal real-world data and safety monitoring mechanisms.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Real-World Safety and Effectiveness analysis sets, comprising all enrolled participants who received ≥ 1 dose of glofitamab. **Sample Size Justification:** Target $N = 300$ provides adequate statistical power to observe precision in real-world rwPFS, CRR, ORR, DCR, safety profiles, and survival estimates within the Chinese adult demographic. **Handling of Missing Data:** Standard handling for observational data, utilizing censoring techniques for time-to-event endpoints and multiple imputation where methodologically appropriate.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Pharmacoepidemiology Practices (GPP), local regulatory requirements for observational registries, and the Declaration of Helsinki. **Monitoring Plan:** Data verification performed through electronic medical record (EMR) abstraction and periodic remote/onsite audits of the participating Chinese clinical centers. **Audit Trail:** Secure eCRF systems utilizing standardized audit trails and data clarification processes.

11. Study Identifiers & Governance

Primary ID: ML45172 **Posting ID:** 884494576106495872 **Registry Number:** NCT07200375 **Sponsor/Lead Organization:** Roche **Study Title:** GlofitReal **Official Regulatory Title:** Evaluating Effectiveness and Safety of Glofitamab Based Second-Line Therapy in Chinese

12. Clinical Framework (DLBCL Specific)

Primary Condition: Diffuse Large B-Cell Lymphoma (R/R DLBCL)
Diagnosis Threshold: Histologically confirmed progression after one prior line of systemic therapy
Medical Methodology: Routine clinical practice administration of Glofitamab (Columvi)
Optimization Target: Tumor response maximization and survival prolongation

13. Study Procedures & Methodology

Management Pathway: Standard clinical oncology pathway for R/R DLBCL in China.
Education Protocol (HCP): Post-marketing standard safety training for bispecific antibodies (e.g., CRS management).
Education Protocol (Patient): Standard counseling regarding expected side effects and monitoring.
Quality Audit Mechanism: EMR-to-eCRF source data verification for real-world evidence integrity.

14. Observation & Follow-Up Schedule

Total Duration: Follow-up continues until death, loss to follow-up, or study termination.
Onsite Visit Cadence: Dictated by standard-of-care treatment and response evaluation cycles (not protocol-mandated).
Remote Monitoring Frequency: Periodic data extraction from institutional EMRs.
Checklist Review Interval: Routine clinical assessment intervals.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				